

UC DAVIS HEALTH

Drug Dialyzability Reference

Conventional HD - High-Flux HD - CRRT

Updated: August 2026 | Includes New & International Drugs

QUICK REFERENCE: WHEN TO SUPPLEMENT

ALWAYS GIVE POST-HEMODIALYSIS:

Aminoglycosides, Vancomycin, Beta-lactams, Gabapentin, Acyclovir, Atenolol, Sitagliptin

NEVER SUPPLEMENT:

Highly protein-bound (Warfarin, Phenytoin), Large Vd (Digoxin, Amiodarone), Hepatically cleared

CONTRAINDICATED IN ESRD:

Metformin, Lithium, Dabigatran, Morphine, NSAIDs, Sulfonylureas

KEY ANTIBIOTICS

Drug	Class	HD	HF-HD	CRRT	Dosing
Amikacin	Aminoglycoside	Y	Y	Y	Give AFTER HD
Vancomycin	Glycopeptide	M	Y	Y	15-20 mg/kg post-HD
Cefepime	Cephalosporin	Y	Y	Y	1-2g post-HD
Meropenem	Carbapenem	Y	Y	Y	1g post-HD
Pip-Tazo	Penicillin+BLI	Y	Y	Y	4.5g post-HD
Levofloxacin	Fluoroquinolone	M	Y	Y	500mg post-HD
Fluconazole	Azole	Y	Y	Y	400mg post-HD daily
Acyclovir	Antiviral	Y	Y	Y	5 mg/kg post-HD

KEY CARDIOVASCULAR MEDS

Drug	Class	HD	HF-HD	CRRT	Dosing
Lisinopril	ACE-I	Y	Y	Y	5-10mg post-HD
Atenolol	Beta-Blocker	Y	Y	Y	25-50mg post-HD
Sotalol	Antiarrhythmic	Y	Y	Y	40-80mg post-HD
Warfarin	Anticoagulant	N	N	N	Monitor INR
Apixaban	DOAC	N	N	N	AVOID if ESRD
Dabigatran	DOAC	M	Y	Y	CONTRAINDICATED

KEY NEUROLOGIC/PSYCH MEDS

Drug	Class	HD	HF-HD	CRRT	Dosing
Gabapentin	Anticonvulsant	Y	Y	Y	100-300mg post-HD
Pregabalin	Anticonvulsant	Y	Y	Y	25-75mg post-HD
Levetiracetam	Anticonvulsant	Y	Y	Y	250-500mg post-HD
Sertraline	SSRI	N	N	N	No supplement
Lithium	Mood Stabilizer	Y	Y	Y	AVOID in ESRD

UC Davis Health - Renal Services
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